HL7® International

June 16, 2025

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The U.S. Department of Health and Human Services (HHS)
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Submitted electronically to:
https://www.regulations.gov/

Re: CMS-0042-NC – Request for Information (RFI): Health Technology Ecosystem

Dear HHS Secretary Kennedy, CMS Administrator Oz and ASTP/ONC Assistant Secretary Keane:

Health Level Seven (HL7) is the global authority on healthcare interoperability and a critical leader and driver in the standards arena. We have partnered with private sector leaders and many federal healthcare agencies including those leading this RFI discussion to drive efficiency and innovation, such as the: Department of Health and Human Services (HHS), Centers for Medicare and Medicaid Services (CMS), and the Office of the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology (ASTP/ONC).

HL7 looks forward to working with HHS, CMS and ASTP/ONC in the healthcare ecosystem arena and to employing out standards and tools, to achieve the RFI goals of:

- Effective and responsible adoption of technology that can empower patients to make better decisions for their health and well-being;
- Improving the state of data interoperability and broader health technology infrastructure; and
- Supporting CMS and ASTP/ONC efforts to lead infrastructure progress, increasing beneficiary access to effective digital capabilities needed to make informed health decisions, and increasing data availability for all stakeholders contributing to health outcomes.

HL7 appreciates and supports the centrality of interoperability, standards-- and HL7 Fast Healthcare Interoperability Resources (HL7® FHIR®) specifically --in this RFI and an emphasis on improving data access and integration while better meeting patient needs. Advancement of HL7 FHIR has been a historical cornerstone of HL7 federal government work. We are pleased it is central to RFI language that calls for better understanding HL7 FHIR-related issues such as:

- Use of FHIR Application Programming Interface (API) and capabilities;
- How increased use of Bulk FHIR could improve use cases and data flow;
- Benefits of using Bulk FHIR for data exports from Electronic Health Records (EHRs) to CMS, simplifying clinical quality data submissions;
- The payer value of a nationwide provider directory that includes FHIR end points and uses digital identity credentials; and
- The potential of combining FHIR addresses and identity to improve data flow.

Our organization has the critical expertise and resources to address the key questions and imperatives of the Health Technology Ecosystem RFI. These include:

- **HL7 Standards Development and Implementation Divisions** – HL7 has well established Standards Development and Standards Implementation Divisions that are key resources**.** The Divisions address industry needs and increase the impact of HL7 standards. The HL7 Standards Development Division focuses on the development and maintenance of HL7 specifications while the Standards Implementation Division concentrates on helping communities discover, access and understand the specifications as well as test their implementations. Please be in touch with the following HL7 executives to address specific standards issues:
    - Daniel Vreeman, DPT; HL7 Chief Standards Development Officer (Dan@hl7.org)
    - Diego Kaminker, HL7 Deputy Chief Standards Implementation Officer (Diego@hl7.org)

- **HL7 Work Groups, Accelerators and Other Efforts -** HL7 Work Groups, FHIR Accelerators and special initiatives are conducting cutting-edge healthcare, interoperability and standards work very relevant to the questions in this RFI. We urge you to seek out the practical and field-leading expertise of these HL7 groups. HL7 executive and policy leaders can also facilitate structured, focused dialogue. A full HL7 Work Group listing can be accessed at: http://www.hl7.org/Special/committees/index.cfm?ref=nav.
  A list of HL7 Accelerators can be found at: https://www.hl7.org/about/fhir-accelerator/,

HL7's specific feedback regarding the CMS RFI: Health Technology Ecosystem is detailed below. Listed in our letter are the RFI questions that are HL7-relevant and our organization's responses. In addition to our leadership and Policy Advisory Committee, HL7 Work Groups and Accelerators contributing to these comments include Patient Empowerment, Payer/Provider Information Exchange, Public Health and the HL7 FHIR FAST Accelerator (FHIR

at Scale Taskforce). We have also included select responses of the HL7 Da Vinci Project and note their full RFI letter, which is available at: https://confluence.hl7.org/spaces/DVP/pages/21857465/Da+Vinci

HL7 has listed in our letter, topical and HL7-aligned recommendations of globally recognized biomedical informatics expert Josh Mandel, MD and these recommendations are explicitly credited and labeled as such. We encourage review of Dr. Mandel's detailed RFI perspectives at: **https://joshuamandel.com/cms-rfi-collab/**

HL7 stands ready to actively assist you in understanding, leveraging and notably boosting deployment of standards and interoperability in the health technology ecosystem. We would welcome further dialogue with you about this topic that is absolutely critical to the future of our nation's healthcare. Should you have any questions about our attached comments, please contact Charles Jaffe, MD, PhD, Chief Executive Officer of Health Level Seven International at cjaffe@HL7.org or 734-677-7777.

Sincerely,

Charles Jaffe, MD, PhD
Chief Executive Officer
HL7 International

Julia Skapik, MD, MPH
Board of Directors, Chair
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HL7 Health Technology Ecosystem RFI Responses

Overarching Comments

Public Health: HL7 emphasizes the critical importance of integrating public health perspectives into the ongoing discussions about the future of the health information technology (IT) ecosystem. More interconnected and patient-centric health technology infrastructure is needed in this area. Public health entities, including state and local health departments, play a pivotal role in disease prevention and control. Their involvement in health IT initiatives ensures that public health data is accurately captured, shared and utilized to inform public health strategies and policies. Public health stakeholders must be included in these conversations to prevent overlooking the broader implications of health IT decisions on community health outcomes. Incorporating public health expertise in health information technology ecosystem talks can help identify potential gaps in data interoperability, ensure that health IT systems support population health management and facilitate timely responses to emerging infectious diseases.

HL7 urges CMS and ASTP/ONC to actively engage public health stakeholders in the development and implementation of health IT policies and systems. By fostering collaboration between healthcare providers, technology developers and public health entities, we can create a more robust health IT ecosystem that serves the needs of all Americans.

Lastly, HL7 recommends infrastructure investments in the public health system to: (1) comply and move to needed new standards; and (2) support the development and deployment of shared standards and content that support meaningful digital exchange. The move towards a cohesive public health data infrastructure and related adoption will likely to take time and be piecemeal-- meaning that some patients and jurisdictions may not get adequate services and data. This could be addressed through centralized infrastructure investments. And, automation of public health data should be emphasized to reduce the cost and human interactions required to support the system.

Section B: Patients and Caregivers
Patient Needs (Patients and Caregivers)

PC-1. What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?

a. What are the top things you would like to be able to do for your or your loved ones' health that can be enabled by digital health products?

b. If you had a personal assistant to support your health needs, what are the top things you would ask them to help with? In your response, please consider tasks that could be supported or facilitated by software solutions in the future.

Integrated, Standards-Supported Apps to Retrieve and Organize Medical Data: HL7 notes the value of a longitudinal record for managing congenital, chronic or complex conditions. The longitudinal patient record allows patients to understand their health, track and manage symptoms, titrate medications, follow long-term care plans and otherwise take responsibility for their health. Apps with the ability to retrieve medical information from various sources and organize it chronologically are also foundational. Longitudinal records and these complex data retrieval and organization tasks are supported by data standards--including HL7 FHIR APIs-- and specifications from the United States Core Data for Interoperability (USCDI). Additionally it is helpful for such information to be integrated, supporting the visualization of a longitudinal patient record that patients themselves can use to communicate their complete medical history to specialty or new providers and to empower them for other activities, such research and generation of real world evidence.

Combined, these tools can connect for enhanced value with other patient-centered apps and devices that collect biometrics, behaviors and signs and symptoms data. HL7 emphasizes the importance of information from patient-centered apps being standardized so that it can be integrated with data from medical providers and other relevant healthcare stakeholders.

PC-2. Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?

HL7 FHIR API Use, Multiple Portals and Personal Health Record (PHR) App Complexity: HL7 observes that the current state of technology often necessitates that a patient or caregiver must utilize multiple portals, instead of a centralized application to access health information. Even though many EHRs have HL7 FHIR APIs enabled, information about how to obtain Personal Health Records (PHRs) apps and their existence is not well known. PHR apps need to also authenticate to each server separately, leading to tedious workflow for the patient and their caregiver. Small medical practices present a special challenge. If a patient has appointments with doctors at small practices, portals and FHIR APIs might not be turned on due to Quality Payment Program (QPP) small practice exceptions. Also, obtaining data from a portal with HL7 FHIR APIs is rare for patients who are in post acute facilities. Easy access to complete health information in one location is currently the exception, not the rule, for most patients and their caregivers. HL7 recommends this be remedied.

Compliance to Patient Access and Information Exchange Requirements:
HL7 recommends that compliance to requirements enabling patient access and exchange could be monitored through testing to determine whether successful exchange occurs. One method for achieving this is by ASTP/ONC requesting audit data regarding: (1) what patients are able to access; (2) what kinds of data actually populate in the application or to the EHR; (3) where there systematic failures of exchange are; and (4) what accessibility testing and features are available.

PC-3. Are you aware of health management, care navigation, or personal health record apps that would be useful to Medicare beneficiaries and their caregivers?

Useful Personal Health Record Apps and PHR Interoperability: HL7 highlights three notable PHR apps: OneRecord, Daylio and MyLinks, which allow patients to centralize, manage and add to all their

data in one place using EHR and payer FHIR APIs. Daylio is a symptom tracker of great interest that has developed notable adoption and support among a wide variety of patients. While not a FHIR compliant app currently, there is a proposed HL7 FHIR Symptoms Data Standards Implementation Guide (IG), that could be used in the future for utilities such as Daylio to share symptom tracking data with healthcare systems. More information on the proposed HL7 FHIR Symptoms Data Standards IG can be accessed at: https://confluence.hl7.org/spaces/FHIR/pages/325467452/Symptoms+Data+Standards+Implementation+Guide

Regarding PHRs, HL7 observes that overall there is not an optimal business model currently for PHR apps. Importantly also, the ability to get data in and out of PHRs is limited by lack of standard interoperability for the PHRs. HL7 is working on the HL7 PHR FHIR IG to address this issue. This IG creates a standardized way for patients to collect and control their medical history in a consolidated digital file. More information about the HL7 PHR FHIR IG can be found at: https://build.fhir.org/ig/HL7/personal-health-record-format-ig/

PC-4. What features are missing from apps you use or that you are aware of today?

a. What apps should exist but do not yet? Why do you believe they do not exist yet?

b. What set of workflows do you believe CMS is uniquely positioned to offer?

Interoperability and Missing Functionality: HL7 notes that missing app functionality may include interoperability points such as the ability to, in a standard way: (1) download the data or knowledge stored or created by the PHR; (2) enable PHR access to a device and smartphone data; (3) upload PHR data to EHRs or communicate with EHRs to schedule appointments or request corrections to a patient record with consistent methods; and (4) communicate about advance directives or to a registry. HL7 is working on the HL7 PHR FHIR IG to address these issues.

Regarding what workflows CMS is uniquely positioned to offer and what is missing from apps, HL7 highlights two important issues: (1) supporting caregiver workflows more robustly and through standard interoperability solutions; and (2) licensing issues. Due to licensing issues, certain coded data downloaded from EHRs cannot be translated on PHRs to human readable text such as CPT-4 codes. These data represent important information such as procedures, so ability to see the human text is needed.

PC-5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

Incentives, Marketing and Exploring New Healthcare Areas: HL7 encourages enhanced marketing and both provider and patient incentives, to encourage patient and caregiver interest in digital health products. Patients empowerment and the tools and strategies that support this, must be at the heart of CMS' digital health product effort. HL7 highlights one additional area of interest, Health Savings Accounts (HSAs) and Flexible Spending Accounts (FSAs). Any device approved for purchase with HSA or FSA funds could have QR and barcodes (SMART Health Link) for example, that provide a

FHIR resource and allow the medication or device to be scanned by a patient's smartphone and imported into their personal health record. This would simply require a database of HSA/FSA approved content and a QR code (SMART Health Link) generator functionality.

Additionally, HL7 highlights the following excerpted recommendations regarding this question, by Dr. Josh Mandel at: https://joshuamandel.com/cms-rfi-collab/.

"Key recommendations to encourage patient interest and optimal digital health product adoption include:

- Mandate API-Accessible, Computable Full EHI Export, Aligning with Industry Specifications
- Keep Single-Patient API Certification Current with SMART App Launch & Backend Services Specifications
- Ensure Programmatic and Automated Access to Medical Images
- Establish a "TEFCA Patient-Developer Credential" for Comprehensive, Direct Data Access
- Empower Individuals with Transparency and Control Over TEFCA Data Sharing
- Ensure Patient Access to Remote, High-Assurance Portal Account Provisioning
- Mandate Electronic Pathways for Patient Record Amendment Requests"

PC-6. What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

SMART Health Links, AI and Usability Modifications: HL7 emphasizes that reducing the amount of work and time it takes for patients and caregivers to be empowered and to utilize tools is key. Helpful strategies for CMS to adopt could include: (1) building in intuitive usability; (2) offering help in choosing trusted/safe apps through certification or consumer reports type functionality; and (3) leveraging evolving capabilities such conversational Artificial Intelligence (AI) with dialog and chat interfaces, that can adjust the grade level to how one speaks and writes.

QR codes (i.e. SMART Health Links) would also provide ease of use by snapping a photo, and achieving connectivity between apps, devices and healthcare systems. This can be established with little or no need for interface configuration.

Data Access and Integration (Patients and Caregivers)

PC-8. In your experience, what health data is readily available and valuable to patients or their caregivers or both?

Patient Contributed Data and EHRs: HL7 notes the need for a standard, definable workflow in order for patient contributed data to enter the EHR. This could provide a more robust way to represent personal caregiver information and contributions as well. The HL7 Patient Empowerment Work Group has composed a white paper on the topic of patient contributed data that can be referenced on this topic. It is available at: https://www.hl7.org/implement/standards/product_brief.cfm?product_id=638.

USCDI and Comprehensive, Patient-Friendly Data Access: Government and private sector stakeholders have steadily expanded the health data readily available to patients and their caregivers, as both digital health and a better understanding of what is needed, has advanced. USCDI is a particularly helpful framework and guidance for such efforts. HL7 appreciates the work on USCDI data elements and HL7 FHIR APIs, but urges more attention be focused on comprehensive and patient friendly data access.

PC-9. Given that the Blue Button 2.0 API only includes basic patient demographic, Medicare coverage, and claims data (Part A, B, D), what additional CMS data sources do developers view as most valuable for inclusion in the API to enable more useful digital products for patients and caretakers?

a. What difficulties are there in accessing or utilizing these data sources today?

b. What suggestions do you have to improve the Blue Button 2.0 API experience?

c. Is there non-CMS data that should be included in the API?

Design, Testing and Longitudinal Records: Regarding additional CMS data sources and enabling more useful digital products, HL7 emphasizes the system should be well designed and tested to ensure support for caregivers who are managing the claims and benefits for a loved one. When constructing longitudinal records, understanding when and where events occur can be of great value in cross-referencing and correlating with other data and understanding environmental effects and causative circumstances. Claims data and other financial receipts are often used as a proxy for understanding symptom onset, proximity circumstances, titration of medications and other matters. In keeping with insurance only aspects of Medicare data, HL7 observes it may be useful to include Location, Encounter, and EpisodeOfCare records in BlueButton 2.0. Claims data is often used to infer these other data points, but having them explicitly available would make the Medicare data that much more useful.

PC-10. How is the Trusted Exchange Framework and Common AgreementTM (TEFCATM) currently helping to advance patient access to health information in the real world?

Technical and Implementation Considerations: Individual Access Services is currently optional due to privacy risk if a search for patients provides another patient. TEFCA is also not implemented fully and especially not connected to small primary practices or long term care. HL7 emphasizes these two issues limit its use. Patient electronic access to small practice information and to long-term care information would be very helpful. HL7 highlights that Clinical Document Architecture (CDA) exchange with IHE XDS< NHIN Direct with CDA, and FHIR with FHIR at Scale Taskforce (FAST) implementation guides are valid solutions in this space.

TEFCA and Data Segmentation: HL7 recommends that over time, data segmentation be rationally integrated into TEFCA implementation and practice to support patient access and trust. HL7 can aid in this migration with FHIR-related standards, tools and mechanisms.

Additionally, HL7 highlights the following excerpted recommendations regarding this question by Dr. Josh Mandel at: https://joshuamandel.com/cms-rfi-collab/.

"TEFCA's impact on *individual patient-initiated access* is still nascent. Its potential requires significant evolution towards patient empowerment.

- **Empower Individuals with Transparency and Control Over TEFCA Data Sharing**: Provide API-accessible audit logs and TEFCA-level patient controls (opt-out, "ask me first," freeze access).

Impactful Use Cases if Implemented Through a Reformed TEFCA (PC-10c):
Patient-initiated aggregation of complete health records; secure sharing with new specialists; individual research with consented data.

Adequate Alternatives Outside TEFCA (PC-10g):
Direct patient access via certified EHR FHIR APIs remains crucial, especially if enhanced by our EHI export recommendations. TEFCA's unique value for querying unknown data holders will only be realized if it fully incorporates patient-centric reforms. Otherwise, direct-to-EHR API access will remain the preferred, more trustworthy pathway for patients."

PC-11. How are health information exchanges (HIEs) currently helping to advance patient access to health information in the real world?

a. How valuable, available, and accurate do you find the data they share to be?
b. What changes would you suggest?
c. Are there particular examples of high-performing HIE models that you believe should be propagated across markets?
d. What is the ongoing role of HIEs amidst other entities facilitating data exchange and broader frameworks for data exchange (for example, vendor health information networks, TEFCA, private exchange networks, etc.)?

HIES and Patient Information Request: HL7 notes HIEs that store information can be a critical, central place patients can request their information. HL7's Patient Empowerment Work Group highlights that AppleHealth and Google CommonHealth are the most widely used vendor HIEs from the patient access API perspective currently, as opposed to state level HIEs. Therefore, government policy regarding HIEs and patient data APIs should recognize this and promote interoperability accordingly.

Information Blocking and Digital Identity (Patients and Caregivers)

Section C: Providers
Digital Health Apps (Providers)

PR-1. What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients?

a. What are the current obstacles?

b. What information should providers share with patients when using digital products in the provision of their care?

c. What responsibilities do providers have when recommending use of a digital product by a patient?

Digital Health Tool Awareness and Assurances: HL7 emphasizes that providers could be encouraged to additionally leverage approved digital health products for their patients by increased awareness of the products and assurances that the products will be safe and effective for their patients (certification, seal of approval, etc.).

PR-3. How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)?
Please address all of the following:
a. Current challenges in accessing different data formats.
b. Impact on patient care quality.
c. Technical barriers to full data accessibility.

Moving Forward on Improving Interoperability in Rural Healthcare Settings: Rural healthcare has distinct considerations. More initiatives should be focused on unique standards and interoperability needs in these settings. HL7 stands ready to assist in this area and to convene or co-convene a special meeting on rural healthcare, interoperability and standards needs. It is critically important for all data in an EHR to be accessible for exchange to ensure patient safety, effective care and cost efficiency. The Agency for Healthcare Research and Quality (AHRQ) has conducted long-standing and insightful work on rural health and both its findings and expertise should be leveraged in these efforts.

PR-4. What changes or improvements to standards or policies might be needed for patients' third-party digital products to have access to administrative workflows, such as auto-populating intake forms, viewing provider information and schedules, and making and modifying an appointment?

HL7 Specific Scheduling, Provider Look-up, Patient Correction Request and FHIR Write Recommendations: The HL7 FHIR Argonaut Accelerator published a scheduling HL7 FHIR IG in the past, but there is no requirement to support it in EHRs or to activate it. Requiring this would allow patients to schedule appointments from their PHRs. More information on the Argonaut Scheduling IG can be found at: https://www.fhir.org/guides/argonaut/scheduling/

A full list of HL7 FHIR Argonaut Accelerator IGs can be accessed at: https://confluence.hl7.org/spaces/AP/pages/91985570/Implementation+Guides.

HL7 asserts that FHIR could be also used to provide patients with provider look up and to support the administrative workflow of requesting a record correction using the Patient Request for Corrections FHIR IG. The HL7 FHIR Argonaut Accelerator has also been working on the concept of FHIR Write - certifying EHRs to support inbound HL7 FHIR workflows from patients (patient contributed data). HL7 believes that requiring provider and payer activation of these workflows, would aid in auto-populating of forms. More information on FHIR Write is at: https://confluence.hl7.org/spaces/AP/pages/345081840/FHIR+Write+-+Notes

Data Exchange (Providers)

PR-11. How could members of trust communities (for example, QHINs, participants and subparticipants in TEFCA, which requires Identity Assurance Level 2 (IAL2) via Credential Service Providers (CSPs)) better support the goals of reduced provider and patient burden while also enhancing identity management and security?

Ensuring Technical Trust and the HL7 FAST Security IG: The RFI identifies the need for end-to-end technical trust between actors in the health data ecosystem--especially in support of TEFCA and secure APIs. HL7 highlights the FAST Security IG which defines the technical mechanisms and policy frameworks necessary to establish and validate mutual trust between organizations. This includes certificate discovery and validation, dynamic metadata exchange, and guidance for how systems determine "who to trust" programmatically. These efforts ensure security can be implemented once and reused across networks, rather than requiring one-off integrations.

PR-12. Should ASTP/ONC consider removing or revising any of the information blocking exceptions or conditions within the exceptions (45 CFR part 171, subparts B through D) to further the access, exchange, and use of electronic health information (EHI) and to promote market competition?

Caution Removing or Revising Information Blocking Exceptions: HL7 recommends caution when considering removal of the above exceptions and to carefully weigh different stakeholder viewpoints. From the patient access perspective, the exceptions provided are needed to help providers keep patients safe and not violate patient privacy.

Section D: Payers

PA-1. What policy or technical limitations do you see in TEFCA? What changes would you suggest to address those limitations? To what degree do you expect these limitations to hinder participation in TEFCA?

HL7 highlights the following excerpted TEFCA limitations and suggested changes by Dr. Josh Mandel at: https://joshuamandel.com/cms-rfi-collab/.

"Policy/Technical Limitations and Suggested Changes:

- **Insufficient Individual Control/Transparency:** Addressed by Empower Individuals with Transparency and Control Over TEFCA Data Sharing to build member trust.
- **Barriers to Innovation for Member-Facing Tools:** Lowered by pathways like Establish a "TEFCA Patient-Developer Credential" for Comprehensive, Direct Data Access.
- **Need for Robust Identity/Authorization:** Supported by Mandate a Trustworthy and Accountable Architecture for All TEFCA Individual Access Services (IAS).
- **Limited Data Scope:** Expand beyond USCDI by enabling exchange of full EHI from systems compliant with Mandate API-Accessible, Computable Full EHI Export, Aligning with Industry Specifications.
- **Lack of Public Foundational Infrastructure:** Addressed by Establish Public Foundational Infrastructure for Nationwide Discovery."

PA-2. How can CMS encourage payers to accelerate the implementation and utilization of APIs for patients, providers, and other payers, similar to the Blue Button 2.0 and Data at the Point of Care APIs released by CMS?

State Prior Authorization Rules: HL7 recommends reconciling and consolidating State Prior Authorization rules that supersede or are duplicative of the Federal CMS-0057-F rule, to clarify expectations, eliminate variances in implementations across state lines and prevent delays from ambiguities of expectations.

Application of Artificial Intelligence (AI): HL7 recommends select, appropriate use of AI to convert complex government regulations and requirements into end-user actionable information.

Payer Implementation Support, Tool Testing and National Sandbox: HL7 supports and emphasizes the following response recommendations of the Da Vinci Project: "to supply payers with tools to test during development at the implementation level will help payers more efficiently build truly interoperable and valuable solutions. A national sandbox to test implementation could provide payers with testing partners early in the process. This will not only ensure development is accurate and efficient, but it can also establish relationships that can support production data exchange with willing partners. Payers need to know that if they are building a FHIR solution, that solution will be used and all other parties necessary to complete the use case will equally be ready to engage. This is an opportunity for CMS and ASTP/ONC to coordinate closely to ensure that all actors engaged in a use case are equally required and/or incentivized to accurately implement the required FHIR solutions.

It is also important that CMS and ASTP/ONC hold payers – and all stakeholders required to implement FHIR solutions – accountable for their implementation and promotion of the completed APIs. There is an opportunity for CMS and ASTP/ONC to support making FHIR API solutions known using incentives and/or disincentives to encourage payers not just build required FHIR API solutions, but encourage and facilitate their use."

PA-4. What would be the value to payers of a nationwide provider directory that included FHIR end points and used digital identity credentials?

HL7 and its Da Vinci Project assert the following:
It is critically important to have a national provider directory to support truly interoperable data exchange and to ensure the investment payers (and providers) are putting into their FHIR data exchange solutions add value for patients. It is a significant barrier to FHIR API implementation not to have such a directory. It will be challenging to scale the use of FHIR without it.

Addressing Provider and Payer Burden, Care Quality and Efficient, Secure, Standards-Based Electronic Transmission of Healthcare Information: Having a national single source of truth will help address three major pain points payers, as well as providers and patients, are experiencing today: (1) it will remove the burden on providers and payers associated with having to update countless directories and/or verify that information in many places often; (2) it will remove the negative impact on care quality that occurs when directories include inaccurate or incomplete information, such as delays in care when a Prior Authorization cannot be efficiently submitted; and (3) it will allow payers and providers to effectively and efficiently transmit secure, standards-based electronic healthcare information across the care continuum where they previously could not due to unknown digital endpoints.

Fundamentals for Payer to Provider and Payer to Payer Data Exchange: Regarding support of payer to provider and payer to payer data exchange, it is important to at a minimum, focus on providers, payers, and their organizational affiliations. Generally, individual providers will not have an endpoint – the endpoint will be an organizational endpoint. It is therefore necessary to understand provider affiliations. To supply enough information for endpoint discovery, and to support finding providers and payers in a national directory, it would also be valuable to include no less than provider demographic information and provider and payer identity information (such as NPI or TIN) to start. Because the National Association of Insurance Commissioners (NAIC) ID is not a truly sufficient payer ID, it would be valuable to also leverage the SMART App Launch "brands" concept that would make it easier for users to know they are identifying the correct payer when working to identify the appropriate endpoints. With this basic information payers (and providers) would have what they need to implement the current CMS and ASTP/ONC interoperability requirements and to take them to scale.

PA-5. What are ways payers can help with simplifying clinical quality data responsibilities of providers?
a. How interested are payers and providers in EHR technology advances that enable Bulk extraction of clinical quality data from EHRs to payers to allow them to do the calculations instead of the provider-side technology?

b. In what ways can the interoperability and quality reporting responsibilities of providers to both CMS and other payers be consolidated so investments can be dually purposed? Are there technologies payers might leverage that would support access to real time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?

HL7 recommends that CMS:

- **De-emphasize Fax Transmittal:** Encourage the removal of fax options to support digital/electronic processes where data can be re-purposed.
- **Spotlight Auditing, Reporting and Compliance Monitoring:** Identify organizations involved in auditing, reporting and compliance monitoring, while also identifying the metrics and requirements of each.
- **Eliminate Duplicative Reporting and Clarify Value of Separate Reporting Streams:** Duplicative reporting occurs anytime Payer coverage reaches across state lines or Medicare Administrative Contractor (MAC) jurisdiction. Billable reporting also requires extra effort and documentation. HL7 recommends: (1) clarifying the value of separate reporting streams when the payer is the same across and within jurisdictions; (2) reducing redundancy of reporting streams across coverage areas; and/or (3) employing better mechanisms of trust to reduce need for reporting streams.
- **Streamline Submission Processes for Redundant Metrics**: Streamline submission processes for redundant metrics (e.g. prior authorization metrics for CMS and the National Committee for Quality Assurance), potentially allowing verified submissions to be re-used for other entities.
- **Evaluate Efficacy of Reporting Requirements**: Evaluate the frequency and complexity of reporting requirements.

Section E: Technology Vendors, Data Providers, and Networks Ecosystem (Technology Vendors, Data Providers, and Networks)

TD-1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

Digital Health Product Business Models, Incentives and Marketing: HL7 observes that many patient facing digital health products currently do not have strong business models. Therefore, incentives would be helpful as a market stimulation, as well as ability to have relevant certified apps and tools well displayed in an easily accessed on-line gallery. Enhanced marketing of digital health product availability to patients and caregivers is also essential.

Overcoming Small Developer Market Barriers: There are significant market barriers to small developers of products. HL7 recommends strategies to address this such as promoting easy access to sandboxes, assured access to marketplaces for products that meet specific certification requirements and approaches to tackle access costs.

HL7 also highlights the following excerpted recommendations regarding this question by Dr. Josh Mandel at: https://joshuamandel.com/cms-rfi-collab/.

"Short-Term Steps (Next 2 Years):

- Advance Mandate API-Accessible, Computable Full EHI Export, Aligning with Industry Specifications.

- Strengthen single-patient FHIR APIs via Keep Single-Patient API Certification Current with SMART App Launch & Backend Services Specifications.
- Launch a pilot for Establish a "TEFCA Patient-Developer Credential" for Comprehensive, Direct Data Access to lower barriers for individual/small developers.
- Commit to and begin developing Establish Public Foundational Infrastructure for Nationwide Discovery.

Longer-Term Steps:

- Ensure TEFCA prioritizes individuals via Empower Individuals with Transparency and Control Over TEFCA Data Sharing and Mandate a Trustworthy and Accountable Architecture for All TEFCA Individual Access Services (IAS).
- Expand certified API capabilities (e.g., Mandate FHIR Subscriptions for Event-Driven Workflows, Mandate CDS Hooks for Seamless Clinical Decision Support Integration).
- Maintain performance parity for standard APIs (Ensure Foundational Design and Performance for Bulk Data API)."

TD-3. Regarding digital identity implementation:

a. What are the challenges and benefits?
b. How would requiring digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs) impact cybersecurity and data exchange?
c. What impact would mandatory use of the OpenID Connect identity protocol have?

FAST Identity and Consent Implementation Guides: As background, the HL7 FHIR at Scale Taskforce (FAST) is an HL7®-affiliated initiative focused on developing scalable, infrastructure-level implementation guides (IGs) and testing frameworks to enable secure, trusted, and nationwide FHIR-based exchange. It conducts critical work in the area of federated digital identity. HL7 highlights that FAST Identity provides a scalable model for establishing federated digital identity across organizational and network boundaries. It leverages established identity assurance levels (IAL/AAL) and supports technical trust through certificate-based authentication. This model enables alignment with TEFCA requirements and allows organizations—especially providers, payers, and state agencies—to verify identities at scale while reducing duplicate efforts and error rates.

This RFI identifies the growing need for interoperable, computable consent frameworks that support patient choice, state variation, legal compliance, and real-time data sharing. HL7 FAST's Consent Implementation Guide will enable organizations to represent, share, and honor consent directives using standardized FHIR-based artifacts, with profiles tailored for HIPAA, 42 CFR Part 2, and TEFCA use cases. It also supports state-specific and multi-program variation—including Medicaid, behavioral health, and social services—while maintaining interoperability and automation, reducing friction and administrative burden in high-need data-sharing environments.

HL7 urges CMS to leverage relevant and innovative HL7 FAST work, particularly in digital identity and consent management and implementation. FAST provides practical, policy-aligned infrastructure solutions that support CMS's goals for modernization, interoperability, and patient empowerment.

FAST's work is implementable, tested and grounded in real-world collaboration across payers, providers, vendors, and government partners.

HL7 urges CMS and ONC to consider the following actions regarding FAST:

- **FAST IG Recognition:** Recognize FAST Implementation Guides and frameworks as aligned with federal policy priorities;
- **Pilot Testing:** Support pilot testing of FAST solutions through CMS Innovation Center models and Medicaid modernization efforts;
- **Certification and TEFCA Guidance**: Align certification and TEFCA implementation guidance with FAST's scalable infrastructure patterns; and
- **Expand Education and Adoption:** Promote industry-wide education and adoption of FAST's identity, consent, security, and directory standards to accelerate interoperability.

A list of HL7 FAST projects and implementation guides can be found at:
https://confluence.hl7.org/spaces/FAST/pages/81009058/FAST+Implementation+Guide+Dashboard
https://confluence.hl7.org/spaces/FAST/pages/130482530/FAST+Projects

Testing and Conformance at Scale: Regarding consistent implementation and testing frameworks for verifying FHIR-based data exchange at scale, HL7 FAST is working with ASTP/ONC-recognized test tools (e.g., Inferno) and industry collaborators (e.g., Sequoia, Drummond) to define a multi-actor, scalable testing framework. This supports cross-organization and cross-use-case testing of Identity, Consent, Security, and Directory and other FHIR Implementation Guides to ensure conformance, promote adoption and prepare implementers for TEFCA-based exchange requirements.

Patient Identification and Public Health: Regarding the public health perspective specifically, HL7 observes that patient identification and matching is a complex but vital piece of public health data management and utilization. HL7 strongly recommends including the public health perspective in deliberations regarding the implementation of digital identity within the healthcare ecosystem. Too often, demographics are either incomplete or inconsistent for definitive patient matching without costly human intervention. Integrating the needs of public health with those of other sectors of healthcare and government will be critical in ensuring that jurisdictions and their trading partners implement digital identity functionality in a standards-based method.

Technical Standards and Certification (Technology Vendors, Data Providers, and Networks)

TD-5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?
A nationwide, free, publicly accessible directory of provider FHIR endpoints is foundational.

Critical National Directory Work (FAST): As stated in the RFI, a reliable, authoritative, and interoperable national directory system is needed to support routing, credentialing, and ecosystem management. HL7 highlights FAST's National Directory work, which provides a blueprint for

a federated system of record capable of queries for healthcare actors (providers, payers, orgs, endpoints), with defined APIs and validation rules for accuracy, provenance, and geospatial semantics. This supports interoperability across CMS programs, and private/public directories, ensuring routing and discovery of services is reliable, efficient, and secure.

HL7 recommends CMS adoption of the FAST National Directory as the foundation for CMS National Directory. The FAST leadership is committed to supporting CMS with the adoption of the Implementation Guide.

Critical National Directory Work (Da Vinci): HL7 and its Da Vinci Project believe that a National Provider Directory is critical to the ability to implement FHIR and achieve interoperability at scale. Generally, individual providers will not have an endpoint – the endpoint will be an organizational endpoint. It is therefore necessary to understand provider affiliations. To provide enough information for endpoint discovery and to support finding providers and payers in a national directory, it would also be valuable to include no less than provider demographic information and provider and payer identity information (such as NPI or TIN) to start. Because the National Association of Insurance Commissioners (NAIC) ID is not a truly sufficient payer ID, it would be valuable to also leverage the SMART App Launch "brands" concept that would make it easier for users to know they are identifying the correct payer when working to identify the appropriate endpoints.

As the HL7 Da Vinci Project notes, a central, federally managed national directory that serves as a central hub that could then supply all downstream directories and systems using FHIR-based APIs is the ideal approach. A centralized directory would allow payers and providers to come to one place and make all needed updates. The efficiencies gained by using one system would support the cost of building it. CMS and ASTP/ONC could also consider factoring the cost into CMS programs and adjusting incentives and disincentives accordingly.

FHIR Endpoints and Doctor/Care Sites: HL7 observes that FHIR endpoints are not enough to directly address a doctor or a specific care sites, which is especially needed for push type notifications and this should be addressed.

TD-6. What unique interoperability functions does TEFCA perform?
a. What existing alternatives should be considered? b. Are there redundant standards, protocols or channels or both that should be consolidated?

HL7 highlights the following excerpted observations and recommendations regarding this question by Dr. Josh Mandel at: https://joshuamandel.com/cms-rfi-collab/.

"TEFCA's *intended* unique function is nationwide querying of unknown data holders under a common trust agreement. Its current realization needs strengthening.

Existing Alternatives (TD-6a):
Direct EHR FHIR APIs (strengthened by Mandate API-Accessible, Computable Full EHI Export, Aligning with Industry Specifications); regional/state HIEs; proprietary vendor networks.

TEFCA should complement, not replace, direct patient-to-EHR API access, offering value for discovery, provided it fully embraces patient empowerment."

TD-7. To what degree has USCDI improved interoperability and exchange and what are its limitations?
a. Does it contain the full extent of data elements you need?
b. If not, is it because of limitations in the definition of the USCDI format or the way it is utilized?
c. If so, would adding more data elements to USCDI add value or create scoping challenges? How could such challenges be addressed?
d. Given improvements in language models, would you prefer a non-proprietary but less structured format that might improve data coverage even if it requires more processing by the receiver?

General: HL7 believes that USCDI has greatly improved interoperability and exchange by providing critical federal guidance and cohesion, as well as clearer signals for healthcare stakeholders and Standards Development Organizations (SDOs). It is a solid overarching framework for progressing forward, however more enhancements and detail must be added. Future USCDI development should also include a focus on practical, real world interoperability and support for an API-accessible, computable full EHI export, which addresses data beyond USCDI.

HL7 also highlights that USCDI annual update frequency creates challenges, including for SDOs like HL7 and in driving change at a pace that industry can fully embrace. More specifically, the rate of USCDI change needs to match industry's capacity to respond and achieve production usage. Overall, the time it takes to integrate USCDI changes and achieve related real implementation success must be acknowledged and reflected in rational policy.

HL7 emphasizes that when USCDI changes are contemplated --including new data elements-- ASTP/ONC should clearly communicate with SDOs about functional expectations, which can inform rational and more directional development of technical specifications and Implementation Guides.

USCDI and Standards-based Repositories: HL7 recommends that USCDI point to standards-based repositories of content and examples when applicable, as opposed to PDFs and written descriptions. HL7 emphasizes that ideally, these should be referenced in FHIR to support data exchange. There is great utility in inclusion of additional metadata over time, as it will support USCDI representation of content in becoming optimal for clinical-decision making and validation across health entities.

Public Health: HL7 highlights that the contents of USCDI meet the basic requirements of many public health program data needs, however, it is important to continue to raise the floor for data availability. By expanding electronic access to all data classes and elements used by public health, the burden and security risks of manual data access will be reduced for both public health and their data submitters. In particular, HL7 recommends having a clear pathway to transition data defined by USCDI+ (particularly the Public Health domain) to USCDI. This will create greater certainty regarding future data access capabilities.

TD-8. What are the most effective certification criteria and standards under the ONC Health IT Certification Program?

Certification, Standards-based Data Sharing and Public Health: The § 170.315(f) requirements supporting data exchange with public health have been instrumental in the development and implementation of standards-based data sharing. HL7 recommends continued expansion of criteria to fully support all public health data needs. We also strongly support coordination between federal agencies to continue to expand incentives, such as Promoting Interoperability, for trading partners such as payers and healthcare organizations to implement real-world connections with public health.

TD-9. Regarding certification of health IT:

a. What are the benefits of redefining certification to prioritize API-enabled capabilities over software functionality?

b. What would be the drawbacks?

c. How could ASTP/ONC revise health IT certification criteria to require APIs to consistently support exchanging data from all aspects of the patient's chart (for example, faxed records, free text, discrete data)?

d. What policy changes could CMS make so providers are motivated to respond to API-based data requests with best possible coverage and quality of data?

e. How could EHRs capable of Bulk data transfer be used to reduce the burden on providers for reporting quality performance data to CMS? What capabilities are needed to show benefit? What concerns are there with this approach?

Value of Health IT Certification and HL7 FHIR Release 4 (FHIR R4) for API Certification Criterion: HL7 believes that there is value in health IT certification in that certification can ensure that health IT products are compliant with relevant standards and support true interoperability. But as the HL7 Da Vinci project emphasizes, it is critical to ensure that health IT certification is not overly burdensome. And, if ASTP/ONC required the use of FHIR R4 for an API certification criterion this would ensure FHIR-based solutions are used that start by leveraging the recommended IGs but would leave the necessary room for the community to more quickly advance the standards and move between versions as needed to ensure adoption and implementation.

Workflows and EHR Data Integration Impact: HL7 highlights the importance of exploring and understanding the impact on workflows that support bringing data into EHRs and into the appropriate clinicians workflow and reconciliation, if certification in this area is not kept. Workflows supporting the collection of data in standardized formats (such as medications with with RxNorm) might also not be maintained.

TD-10. For EHR and other developers subject to the ONC Health IT Certification Program, what further steps should ASTP/ONC consider to implement the 21st Century Cures Act's API condition of certification (42 U.S.C. 300jj-11(c)(5)(D)(iv)) that requires a developer's APIs to allow health information to be accessed, exchanged, and used without special effort, including providing access to all data elements of a patient's electronic health record to the extent permissible under applicable privacy laws?

HL7 and its Da Vinci Project assert:

The ultimate goal is to have the full patient's record available so that the right data can be available at the right time in the right place. If the full patient's record is available, a payer could access the portion they need, a provider the portion they need, and digital health products the portions necessary to meet user needs and product goals. For this to be most valuable, all data in the patient's record would be FHIR enabled and standardized. We are not yet there. Only a portion of the data, largely that defined within the USCDI, is standardized. But, that is a valuable start. HL7 believes that ASTP/ONC should continue to ensure the certified health IT is able to share the full patient record even if that is currently shared as largely unstructured data through the EHI export. This is an initial step.

Structured Patient Records: Ultimately, it is desirable that only the minimal data needed to best serve the patient and meet the use case at hand is accessed. To do this, a fully structured patient record is needed. This is incrementally achieved by starting with the USCDI – the structured subset of the patient record – and EHI export to share via FHIR unstructured data not yet mapped to FHIR. As the structured portion of the record increases, there should be less need for EHI export over time.

Constraints and Considerations: As with many use cases, the main constraints to date for large data exchange are issues with compute resources that have limited Bulk data transfers, as well as issues with consent and attribution when looking to share large amounts of data across a panel of patients. ASTP/ONC should continue to support industry in working through concerns and considerations around consent and CMS. HL7 urges ASTP/ONC to continue to support the HL7 community in addressing attribution and Bulk implementations to help facilitate this data exchange.

Additionally, HL7 highlights the following excerpted recommendations regarding this question by Dr. Josh Mandel at: https://joshuamandel.com/cms-rfi-collab/.

"The Cures Act's vision of data being accessed, exchanged, and used "without special effort" extends beyond simple retrieval. It encompasses the full lifecycle of patient interaction with their data, including ensuring its accuracy and completeness.

Primary Technology Policy Recommendation:

- Mandate API-Accessible, Computable Full EHI Export, Aligning with Industry Specifications: This is precisely designed to fulfill the Cures Act's "all data elements ... without special effort" provision by requiring API accessibility, inclusion of all EHI, and computability via vendor documentation.

Further Supporting Recommendations ensuring "without special effort" for access and use:

- Keep Single-Patient API Certification Current with SMART App Launch & Backend Services Specifications:
- Ensure Programmatic and Automated Access to Medical Images
- Mandate Electronic Pathways for Patient Record Amendment Requests: Fulfilling the HIPAA right to request amendment "without special effort" is a crucial aspect of "using" one's health information. Current manual processes create significant burdens. Mandating certified electronic pathways (via portals and APIs) for patients to submit, track, and receive responses to amendment requests directly aligns with the Cures Act's intent to empower patients and improve data quality.

By mandating these capabilities through the Health IT Certification Program, ASTP/ONC can ensure that "without special effort" becomes a practical reality for patients seeking to truly engage with and manage their complete health information."

TD-11. As of January 1, 2024, many health IT developers with products certified through the ONC Health IT Certification Program are required to include the capability to perform an electronic health information export or "EHI export" for a single patient as well as for patient populations (45 CFR 170.315(b)(10))

a. Should this capability be revised to specify standardized API requirements for EHI export? b. Are there specific workflow aspects that could be improved? c. Should CMS consider policy changes to support this capability's use?

Standardization, Full Functionality and the HL7 Argonaut Project's EHI Export API IG: Yes, the EHI export capability should be revised to specify standardized API requirements. HL7 observes that the functionality is limited without standardization. Although it is a significant effort to fully standardize API requirements for EHI export, the goal should be to move towards full standardization. HL7 recommends that ASTP/ONC should require alignment with or equivalence to the HL7 Argonaut Project's EHI Export API IG that defines a standards-based API for accessing a patient's complete electronic health record, compliant with the 21st Century Cures Act and leverages HL7 FHIR technology for patient access and sharing of their EHI.

Section F: Value-Based Care Organizations
Digital Health Adoption (Value-Based Care Organizations)

VB-3. What are essential health IT capabilities for value-based care arrangements?

a. Examples (not comprehensive) may include: care planning, patient event notification, data extraction/normalization, quality performance measurement, access to claims data, attribution and patient ID matching, remote device interoperability, or other patient empowerment tools.

b. What other health IT capabilities have proven valuable to succeeding in value-based care arrangements?

Value-based Care Models and Specifications: Currently, CMS does not provide full specifications for value-based care models. HL7 highlights that this leads to fragmentation regarding how participants can understand requirements, monitor their progress and hold CMS accountable for payment adjustments. HL7 emphasizes that this must be corrected with CMS requiring fully specified models (FHIR would be one option) that include value sets and clear definitions for model inclusion, exceptions and adjustments.

HL7 and its Da Vinci Project's put forth the following observations and recommendations:

Essential Health IT Capabilities for Value-Based Care Arrangements: As an HL7 Accelerator, the Da Vinci Project is responsible for supporting industry, specifically providers and payers, but also critical vendor partners, to develop and implement value-based care standards. We believe that to achieve CMS' value-based care goals, it is necessary to leverage FHIR-based APIs. FHIR-based APIs can support Accountable Care Organizations (ACOs) and any value-based care arrangement with foundational work from supporting patient care to quality measurement. FHIR-based APIs can both facilitate efficient care and support automated analysis of patient outcomes, utilization, and expenditures.

Digital Health IT, Data Availability and APIs: Digital health IT can support value-based care by ensuring the right data are in the right place at the right time through use case specific APIs. ACOs could benefit from leveraging both the Provider Access API and Payer-to-Payer API to ensure the patient's record is available for care delivery support and effective care coordination. The Prior Authorization API can support more timely care and care plan adherence. A FHIR-based national directory leveraging a national digital identity could ensure the right records are shared with the right entities efficiently to ensure maximum value and benefit from FHIR API use. ACOs already have access to Medicare claims data via the Beneficiary Claims Data API (BCDA). Access to claims data via APIs like BCDA and Data at the Point of Care (DPC) could further support access to claims data, which can support utilization and expenditure monitoring and evaluation on FHIR.

Patients would benefit from access to Blue Button/Patient Access API-fueled apps so they could monitor their own health data and be true partners in their value-based care journey. There are many opportunities for value-based care arrangements to leverage FHIR and FHIR-supported digital health IT, and doing so is necessary if value-based care is going to truly scale to all patients. These programs also offer an opportunity for CMS to provide direct support and/or incentives to help ACOs build the needed digital infrastructure and adopt digital health technology more broadly and effectively.

Additionally, HL7 highlights the following excerpted recommendations regarding this question by Dr. Josh Mandel at: https://joshuamandel.com/cms-rfi-collab/.

"Essential Health IT Capabilities Supported by Our Recommendations:

- **Efficient Data Extraction/Aggregation:** Keep Bulk Data API Certification Current with FHIR Bulk Data Specifications, Ensure Foundational Design and Performance for Bulk Data API,

and Mandate API-Accessible, Computable Full EHI Export, Aligning with Industry Specifications.

- **Timely Patient Event Notifications:** Mandate FHIR Subscriptions for Event-Driven Workflows.
- **Advanced CDS/Workflow Integration:** Mandate CDS Hooks for Seamless Clinical Decision Support Integration.
- **Comprehensive Data for Quality Measurement:** Steward USCDI Development for Pragmatic Interoperability and Bulk FHIR capabilities.
- **Enhanced Patient Engagement:** Patient data access through Mandate API-Accessible, Computable Full EHI Export, Aligning with Industry Specifications and Keep Single-Patient API Certification Current with SMART App Launch & Backend Services Specifications.
- **Nationwide Data Discovery:** Establish Public Foundational Infrastructure for Nationwide Discovery and a reformed TEFCA (e.g., per Empower Individuals with Transparency and Control Over TEFCA Data Sharing)."